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slatehealth

Business Proposal & Market Opportunity

AI Agents That Replace Healthcare Administrators

$496B	4,800%	10:1	$22B+
Annual Admin Waste	Admin Growth	Admin-to-Doctor	Revenue TAM

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SECTION 01

Executive Summary

Slate Health deploys AI agents that replace healthcare administrators — the fastest-growing, least productive cost center in American medicine.

> **Core Thesis:** Healthcare managers have grown 4,800% since 1970 while physicians grew just 200%. The US spends $496 billion per year on billing and insurance administration alone — more than the GDP of most countries. AI can now automate 70%+ of these tasks. Slate captures this opportunity by charging 15% of the labor it replaces, creating a $22B+ revenue TAM with zero-risk adoption for buyers.

$1T
Annual US healthcare admin costs

$248B
Excess unnecessary admin waste

70%
Admin tasks automatable by AI (McKinsey)

$22B+
Slate revenue TAM (US only)

What We're Building

Slate Health is an AI-native platform that deploys six specialized agents to handle the core functions of healthcare administration: prior authorization, claims & billing, scheduling, eligibility verification, credentialing, and compliance reporting. Each agent replaces an entire department. They run 24/7, process thousands of transactions per hour, and improve continuously.

Business Model

Performance-based pricing: Slate charges **15% of the admin labor it replaces**. Health systems keep 85% of the savings. If Slate doesn't deliver measurable savings, clients pay nothing. This eliminates adoption risk and aligns incentives directly with outcomes.

Market Timing

AI Capability Inflection: Healthcare AI market is $37B in 2025, projected $500B+ by 2034 (37% CAGR). AI now handles 70%+ of revenue cycle tasks autonomously. 54% of all digital health VC funding in 2025 went to AI companies.

Regulatory Tailwinds: CMS electronic prior authorization mandates take effect 2027. Hospital admin expenses doubled from $367B (2011) to $687B (2023). Health systems are desperate for cost reduction.

5-Year Trajectory at a Glance

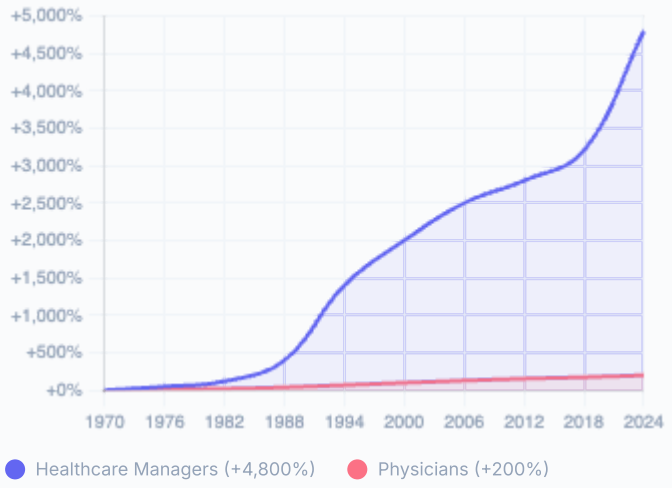
	YEAR 1	YEAR 2	YEAR 3	YEAR 4	YEAR 5
ARR	$1.2M	$8.4M	$36M	$124M	**$333M**
Customers	8	42	138	400	950
FTEs Replaced	200+	2,000+	12,000+	40,000+	100,000+
Exit Valuation	—	—	Series B	IPO-ready	**$5–8B**

SECTION 02

Market Problem: The Healthcare Admin Crisis

Since 1970, American healthcare added administrators 24x faster than physicians. The bureaucracy is now the largest cost center in the system.

Growth of Healthcare Managers vs. Physicians (1970–2024)



Source: Current Population Survey, Bureau of Labor Statistics & National Center for Health Statistics (Himmelstein & Woolhandler)

$1.1T
Total annual admin costs — more than Indonesia's GDP

10 : 1
Administrators per physician in US healthcare

5x
US admin spend per capita vs. peer nations ($1,078 vs ~$200)

2.7%
Canada's admin rate vs. 15-25% in the US

Where the $496B Goes

$83B
Routine admin transactions. Eligibility, claims, prior auth, payment posting. Providers bear 97%.

$150B
No-show losses. 18-23% no-show rate. AI scheduling cuts this 50-70%.

$38.6B
Compliance costs. 59 FTEs per hospital on compliance. $7.6M/year each.

The inefficiency is staggering: 11.8% of claims are denied. 81.7% are overturned on appeal. Each denial costs $57 to rework. Prior auth alone consumes 14 hours/week per physician practice. The system generates $248B in excess, unnecessary administrative costs every year.

SECTION 03

Product: Six AI Agents

Each Slate agent replaces an entire administrative department. They operate 24/7, process thousands of transactions per hour, and compound in accuracy daily.

Prior Authorization Agent

Submits, tracks, and appeals prior authorizations. Gathers clinical documentation, monitors approval status, manages renewals and expirations automatically.

14 hrs/week saved per practice | 81.7% appeal win rate

Claims & Billing Agent

Codes encounters (ICD-10, CPT, HCPCS), submits claims, posts payments, catches errors pre-submission, manages denials and files appeals.

$57/denial eliminated | 40-60% denial reduction

Scheduling & Access Agent

Manages appointments, waitlists, referral coordination, and no-show prediction. AI outreach reduces no-shows 50-70%. 24/7 patient access.

$150B no-show market | 24/7 availability

Eligibility Verification Agent

Real-time insurance verification, benefits confirmation, network status checks, and multi-plan coordination of benefits. Instant results.

$14 → <$1 per check | 93% faster

Credentialing Agent

Provider credentialing and payer enrollment in days, not months. CAQH profile management, license renewals, malpractice verification.

90 → 14 days | $3,500 → $200/provider

Compliance & Reporting Agent

Continuous HIPAA monitoring, quality measure reporting (HEDIS, MIPS, CMS Stars), documentation audits, regulatory compliance tracking.

59 FTEs → 3 FTEs | $7.6M saved per hospital

Deployment Model

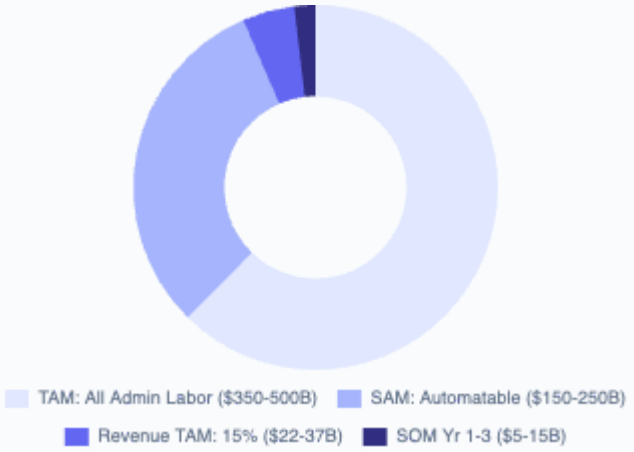
1. **Connect** — FHIR, HL7, and direct API integration with existing EHR and PM systems
2. **Learn** — 2-week observation of payer rules, coding patterns, and org preferences
3. **Deploy** — Human-in-the-loop oversight while agents ramp accuracy on live tasks
4. **Automate** — Fully autonomous 24/7 operations. Human supervisors handle exceptions only.

SECTION 04

Total Addressable Market

Slate targets the largest cost center in American healthcare. At a 15% take rate on automatable labor, the US revenue TAM exceeds $22 billion.

TAM / SAM / SOM Framework



LAYER	SCOPE	SIZE
TAM	All US healthcare admin labor	**$350–500B**
SAM	Automatable admin tasks (50-70%)	**$150–250B**
Revenue TAM	Slate's 15% of SAM	**$22–37B**
SOM (Yr 1-3)	Hospital systems + mid-size practices	**$5–15B**

US Healthcare Facilities

FACILITY TYPE	COUNT
Hospitals	**6,100**
Health systems / IDNs	**576–800+**
Physician practices	**395,000**
Ambulatory Surgery Centers	**12,294**

Admin Spend by Facility Type

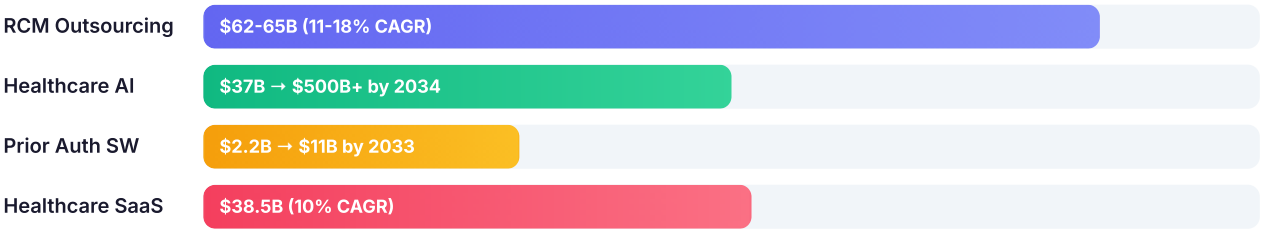
$29.4M / hospital

Average annual admin expense per hospital. 300-370 admin FTEs each. Hospitals have more billing specialists than beds.

$150K–$500K / practice

Admin spend per physician practice, varying by size. 60-70% of practice revenue goes to overhead; half is staff costs.

Adjacent Market Sizes



SECTION 05

Competitive Landscape

The market has established incumbents and well-funded AI challengers. Slate's performance-based model and full-stack agent approach differentiates from all of them.

COMPANY	FOCUS	REVENUE / FUNDING	VALUATION	STATUS
Waystar	Cloud RCM platform	$1.1B revenue; IPO ($968M)	**$4.7B mkt cap**	Public (WAY)
Thoughtful AI	AI agents for RCM	**$100M ARR** (2024)	Private	Active — closest comp
AKASA	AI RCM automation	$259M raised (Series C)	Not disclosed	Active
Cohere Health	AI prior authorization	$200M raised (Series C)	Not disclosed	Active
Cedar	Patient billing / engagement	$350M raised (Series D)	**$3.2B**	Active
Hippocratic AI	Healthcare AI agents	$404M raised (Series C)	**$3.5B**	Active
symplr	Healthcare ops (credentialing, compliance)	9/10 US hospitals	PE-backed	Incumbent — fragmented UX
Olive AI	RCM automation	$902M raised; **shut down**	$4B peak → $0	Dead (Oct 2023)

Key insight: Thoughtful AI proved $100M ARR is achievable in AI-for-admin. But most competitors focus on a single vertical (Cohere = prior auth, Cedar = billing). Slate's advantage is the **full-stack, six-agent approach** — one platform replaces the entire admin layer, not just one function. Combined with performance-based pricing (vs. SaaS seat fees), Slate de-risks adoption and captures more wallet share per customer.

Slate's Competitive Differentiation

DIMENSION	INCUMBENTS
Scope	Single function (billing OR auth OR credentialing)
Pricing	SaaS per-seat or per-transaction fees
Risk	Buyer bears implementation risk
Architecture	Rules-based or narrow ML models

DIMENSION	SLATE HEALTH
Scope	**Full admin stack — 6 agents, all functions**
Pricing	**15% of labor replaced (outcome-based)**
Risk	**$0 if no savings — zero buyer risk**
Architecture	**Agentic AI (LLM-powered autonomous agents)**

SECTION 06

Business Model & Pricing

Slate aligns revenue directly with customer outcomes. We earn money only when our customers save money.

15%
Our fee. Of the admin labor Slate replaces. Tied to measurable outcomes.

85%
Client keeps. Pure bottom-line savings from month one.

$0
Performance guarantee. No savings = no payment. Zero risk.

Example: Mid-Size Hospital System (64 admin staff)

LINE ITEM	AMOUNT
Current admin labor spend (64 FTEs × $72,800)	**$4,659,200**
Automatable portion (100% of tasks selected)	$4,659,200
AI automation rate (85%)	$3,960,320
Slate platform cost (15% of savings)	($594,048)
Net savings to hospital	**$3,366,272**

Impact Metrics (Same Example)

54
FTEs Replaced

5.7×
ROI Multiple

64 days
Payback Period

112K
Hours Freed / Year

Why Performance-Based Wins

1. Zero-risk adoption. CFOs don't need to justify upfront spend. If it doesn't work, they pay nothing. This compresses sales cycles from 12-18 months to 3-6 months.

2. Higher ACV over time. As agents automate more tasks, the labor-replaced increases, and so does revenue — without upselling.

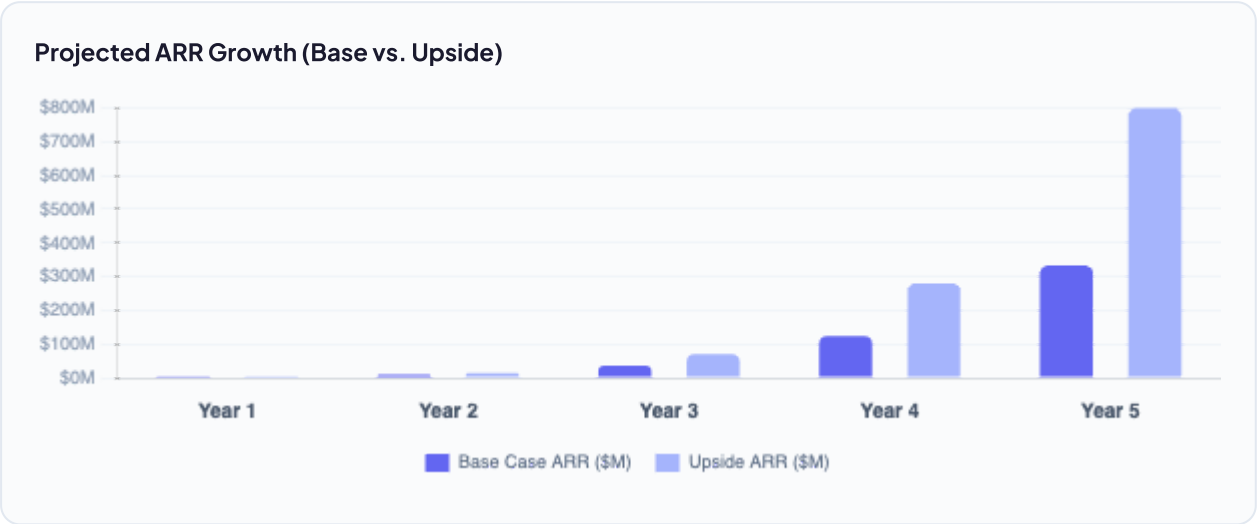
3. Sticky by design. Once agents are processing claims and prior auths, switching costs are high. Net revenue retention should exceed 130%.

4. Land and expand. Start with prior auth + billing (50% of admin time), expand to all six agents over 12 months.

SECTION 07

Financial Model: 5-Year ARR Projections

Conservative base case with upside scenario. Assumes performance-based pricing at 15% of labor replaced, 6-12 month sales cycles, and land-and-expand motion.



Base Case Financial Model

	YEAR 1	YEAR 2	YEAR 3	YEAR 4	YEAR 5
New Customers	8	35	100	280	600
Total Customers	8	42	138	400	950
Avg ACV	$150K	$200K	$260K	$310K	$350K
Gross Churn	10%	8%	6%	5%	5%
Net Revenue Retention	110%	125%	135%	130%	125%
ARR	**$1.2M**	**$8.4M**	**$36M**	**$124M**	**$333M**
YoY Growth	—	600%	329%	244%	169%

Upside Scenario

	YEAR 1	YEAR 2	YEAR 3	YEAR 4	YEAR 5
Total Customers	12	60	250	800	2,000
Avg ACV	$170K	$220K	$280K	$350K	$400K
ARR	**$2.0M**	**$13.2M**	**$70M**	**$280M**	**$800M**

Benchmark: Thoughtful AI reached $100M ARR in ~3 years doing AI agents for RCM. Waystar generates $1.1B revenue serving 30,000 clients with traditional cloud RCM software. Both validate the growth trajectory. With performance-based pricing and a full-stack agent approach, Slate is positioned to grow faster.

SECTION 08

Unit Economics

At scale, Slate's unit economics are exceptionally attractive: AI agents cost a fraction of the labor they replace, creating high gross margins and strong LTV/CAC ratios.

Key Unit Economics (at scale)

Average ACV (Year 3)	**$260,000**
Gross Margin	**85-90%**
AI Compute Cost per Customer	~$2,000-5,000/mo
CAC (blended)	$40,000-60,000
Payback Period	3-4 months
Net Revenue Retention	**125-135%**
LTV (5-year)	**$1.2M+**
LTV / CAC	**20-30x**
Gross Churn	<6% (at scale)

Why Margins Are High

Near-zero marginal cost. Each AI agent processes thousands of transactions per hour. The compute cost to handle one hospital's prior auths is ~$500/month. The labor it replaces costs $50,000+/month. That's a 99% cost advantage.

Expand without selling. As agents automate more tasks within an account, revenue grows automatically (more labor replaced = higher 15% fee). NRR exceeds 125% without requiring sales effort on existing accounts.

Cost of Replacement: AI Agent vs. Human Administrator

METRIC	HUMAN ADMIN	SLATE AI AGENT	DELTA
Annual cost (fully loaded)	$72,800	$3,600	**20x cheaper**
Hours available / year	2,080	8,760	**4.2x more**
Transactions / hour	4-8	500+	**60-125x faster**
Error rate	5-10%	<1%	**5-10x fewer errors**
Sick days / PTO	15-25 days	0	**100% uptime**

SECTION 09 & 10

Go-to-Market Strategy

Land with prior auth + billing (50% of admin time, highest pain), expand to all six agents over 12 months.

Phased GTM Plan

PHASE	TIMELINE	FOCUS	TARGET SEGMENT
1. Prove	Months 1-6	Land 5-8 pilot customers. Demonstrate ROI on prior auth + claims agents. Build case studies.	Mid-size health systems (50-200 beds)
2. Sell	Months 7-18	Hire 4-6 AEs. Launch all 6 agents. Target CFOs and COOs directly. Series A.	Health systems + large practices
3. Scale	Months 19-36	Channel partnerships (EHR vendors, consultancies). Enterprise sales team. Series B.	Large IDNs, hospital chains
4. Dominate	Months 37-60	Category leader. Compete head-to-head with RCM outsourcers (R1, Optum). International expansion.	All US facilities + international

Sales Motion

Lead with the ROI calculator. The homepage calculator is the top-of-funnel. CFOs input their numbers, see 5-7x ROI instantly. Converts to a "custom ROI report" lead magnet.

Sell to the CFO, not IT. This is a cost-reduction play, not a technology play. The buyer is the person who signs checks for admin staff — the CFO or COO.

Performance guarantee closes deals. "$0 if no savings" eliminates the procurement objection. Legal and compliance review is faster when there's no financial commitment upfront.

Expand automatically. Once prior auth + billing agents are live, scheduling and eligibility agents deploy naturally. Revenue grows without upsell effort.

Risk Analysis

RISK	SEVERITY	MITIGATION
Long enterprise sales cycles	Medium	Performance guarantee ($0 risk) compresses cycles. Target mid-size first.
HIPAA compliance complexity	High	SOC 2 Type II + HIPAA BAA from day one. No PHI stored — agents process in transit.
Incumbent response (Waystar, R1, Optum)	Medium	Incumbents are adding AI features, not rebuilding as AI-native. Speed is our moat.
AI accuracy / hallucination risk	High	Human-in-the-loop for first 30 days. Continuous accuracy monitoring. Audit trails on every transaction.
Olive AI cautionary tale	Medium	Olive failed from overexpansion and loss of focus. Slate stays disciplined: 6 agents, one market, performance-based.
Regulatory changes	Low	CMS mandates favor automation (e-prior auth 2027). Regulation is a tailwind, not a headwind.

Exit Potential (Year 5): At $333M ARR with 85%+ gross margins and 125%+ NRR, Slate would be valued at **15-25x ARR = $5-8B** for an IPO or strategic acquisition. Potential acquirers include UnitedHealth Group, Oracle Health (Cerner),

Epic, or large PE firms active in healthcare IT.

SECTION 11

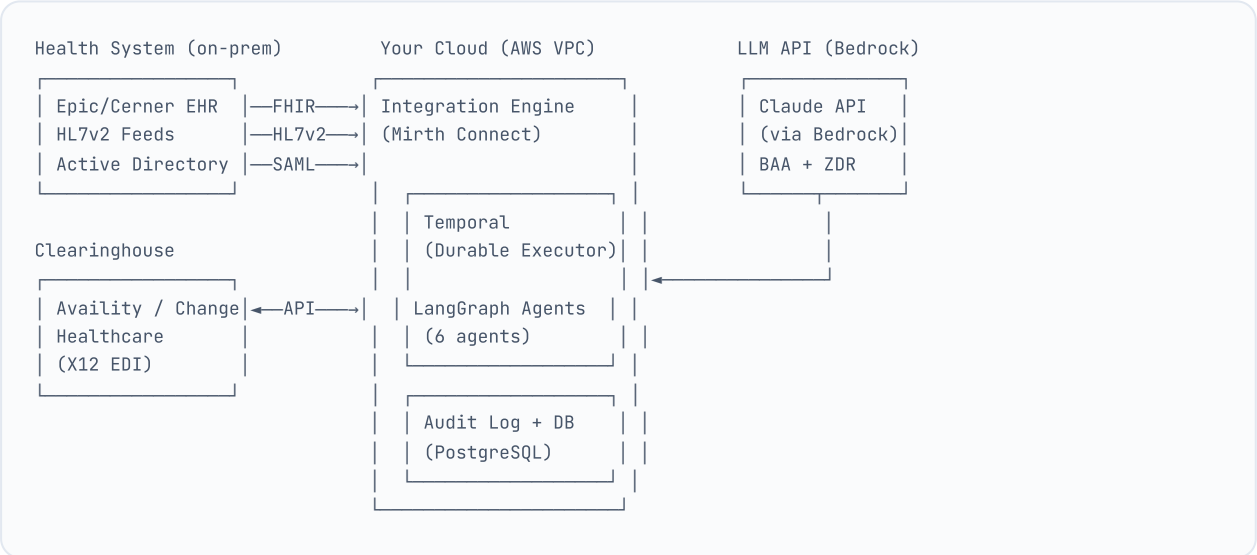
Technical Architecture

Production stack for deploying AI agents in HIPAA-compliant healthcare environments. Cloud-first with BAA coverage — on-premise is not required.

Recommended Framework Stack

LAYER	TOOL	WHY
Agent Orchestration	**LangGraph**	Graph-based stateful workflows. Self-hosted Enterprise keeps PHI in VPC. Model-agnostic. Every decision path is an inspectable, replayable graph node.
Durable Execution	**Temporal**	Guarantees workflows run to completion despite failures. Self-hosted, open source. Every step persisted, auditable, recoverable.
LLM Backend	**Claude via AWS Bedrock**	Anthropic offers BAAs. Bedrock = HIPAA-eligible infra with zero data retention. No PHI stored or used for training.
Integration Engine	**Mirth Connect**	Open-source. Translates between FHIR, HL7v2, and X12 EDI. The glue between agents and health system infrastructure.
Payer Connectivity	**Availity / Claim.MD**	Clearinghouse APIs that abstract X12 EDI. Startup-friendly onboarding. Don't build raw payer integrations.

System Architecture



HIPAA Compliance: BAA Availability

PROVIDER	BAA	COVERED SERVICES	NOT COVERED
Anthropic	Yes	Claude API with Zero Data Retention; Enterprise plan	Free/Pro/Max/Team plans
AWS	Yes	Bedrock (all models), Bedrock Agents, AgentCore	Non-HIPAA-designated accounts
Microsoft Azure	Yes	Azure OpenAI Service, Azure AI Foundry	Consumer services
Google Cloud	Yes	Vertex AI, Gemini (with HIPAA project flag)	Google AI Studio, consumer Gemini
OpenAI	Yes	API with Zero Data Retention only	ChatGPT Free/Plus/Pro/Team

On-premise is NOT required. Cloud with a BAA is the mainstream approach used by AKASA, Thoughtful AI, Cohere Health, and every major healthcare AI company. For maximum-security customers, deploy in the customer's own AWS/Azure account under their BAA. Open-source models (LLaMA 3, Gemma) via NVIDIA NIM are available for complete data isolation, with a quality trade-off.

SECTION 12

Agent Build Order & Shared Chassis

All 6 agents share a core chassis representing ~60% of the engineering work. Build it once, deploy all agents on top. Ranked from easiest to hardest.

Agent Difficulty Ranking

#	AGENT	DIFFICULTY	KEY CHALLENGE	BUILD TIME
1	**Eligibility Verification**	Low	Well-standardized (X12 270/271). Clearinghouse APIs handle it cleanly. Minimal judgment.	**4-6 wks**
2	**Scheduling & Access**	Low-Med	EHR scheduling APIs exist (FHIR Appointment). AI layer is NLP + optimization. No payer interaction.	**6-8 wks**
3	**Credentialing**	Medium	Structured but long (90+ day) process. CAQH has APIs. More workflow automation than AI reasoning.	**8-10 wks**
4	**Compliance & Reporting**	Medium	Rule-based at core (HEDIS, MIPS are published specs). AI adds document analysis. Internal-facing = lower risk.	**8-10 wks**
5	**Claims & Billing**	Hard	Medical coding (ICD-10, CPT) requires deep clinical understanding. Near-zero error tolerance. Complex denial loops.	**12-16 wks**
6	**Prior Authorization**	Very Hard	Every payer has different rules/forms. No universal API until 2027. Needs clinical judgment + RPA fallback.	**16-20 wks**

Key insight: The hardest agent (Prior Auth) is the most valuable — 14 hrs/week per practice, $248B in excess costs. Cohere Health raised $200M on it alone. Strategy: **start with Eligibility** (prove the platform) but **invest heavily in Prior Auth** (where the money is).

Shared Agent Chassis (60% of engineering)

FHIR/HL7 Data Ingestion

Read patient/encounter context from EHR via standardized interfaces. Canonical data model maps all formats.

LLM Reasoning Layer

LangGraph + Claude. Understands tasks, makes decisions, handles edge cases. Graph-based for auditability.

Deterministic Action Execution

LLM decides WHAT to do. Validated code executes HOW. No free-form LLM output touching production systems.

Human-in-the-Loop Escalation

Confidence thresholds route uncertain decisions to human review. All denials involve human oversight.

Audit Trail & Compliance

Every decision, API call, and data access logged to immutable store. Full replay capability for any transaction.

Payer Rule Engine

Each payer has different rules. Agents reference a continuously updated rules database. Versioned and auditable.

SECTION 13

Integration Playbook & Phased Build Plan

The hardest part isn't the AI — it's the integration slog and health system procurement cycle. Here's how to move fastest.

Integration Timelines

INTEGRATION	TIMELINE	WHO DOES IT	SELF-SERVICE?
SSO (SAML/OIDC)	2-4 weeks	Health system IT	Yes
Eligibility via clearinghouse	1-2 months	Your eng + their billing	Partial
Claims via clearinghouse	2-3 months	Your engineers	No
FHIR read from Epic/Oracle	3-6 months	Your eng + their IT	No
FHIR write-back to EHR	6-12 months	Your eng + their IT	No
Prior auth (multi-payer)	3-6 months	Your engineers (ongoing)	No
Full 6-agent deployment	6-12 months	Embedded on-site	No

EHR Priority Order

1st
athenahealth
Cloud-native, best APIs, 3-6 mo integration. Start here.

2nd
Epic
50%+ market share. 6-12 mo integration. Begin App Market review in Month 1.

3rd
Oracle Health
FHIR R4 via Ignite APIs. 6-12 mo. More open than Epic historically.

4th
MEDITECH
Limited APIs (older installs). HL7v2 only. 6-18 mo. Last priority.

12-Month Phased Build Plan

PHASE	MONTHS	DELIVERABLES	TEAM
1. Foundation	1-3	Shared chassis (LangGraph + Temporal + Bedrock). **Eligibility Agent** live. Availity connected. SSO. 2-3 athenahealth pilots. Epic App Market review started.	4-6 eng
2. Expand	3-6	**Scheduling Agent** + **Claims Agent** live. Change Healthcare as 2nd clearinghouse. Human review dashboard. 8-10 customers. HIPAA BAA executed.	6-8 eng
3. Monetize	6-9	**Prior Auth Agent** live (the big one). Portal automation (RPA). Da Vinci PAS support. Epic approval. First Epic customer. **Series A.**	8-12 eng
4. Full Platform	9-12	**Credentialing** + **Compliance** agents live. First hospital system (all 6 agents). Oracle Health integration. Published case studies. 20-30 customers. **~$5M ARR.**	10-15 eng

Speed multipliers: (1) Start with athenahealth — 3x faster integration than Epic. (2) Use Availity/Claim.MD — fastest clearinghouse onboarding. (3) Build integration engine once with Mirth Connect — each new customer is config, not code. (4) Build Da Vinci PAS now — be ready when CMS mandate hits 2027 and competitors scramble.

SECTION 14

Prior Authorization Deep Dive

PA is a $98B administrative cost center with only ~3% software penetration — one of the largest services-to-software conversion opportunities in any industry. The procedural/imaging subcategory is the hardest, highest-value, and least-automated segment.

$98B
PA-related admin spend. ~3% addressed by software today.

$15–20B
PA automation TAM by 2027 (Waystar SEC filings)

45/week
Avg PAs completed per physician per week

$15B
CMS-estimated savings over 10 years from FHIR PA mandate

Why PA Is Hard to Automate

Payer Fragmentation

Every payer — UHC, Aetna, Cigna, each of 33+ BCBS plans, Medicaid MCOs — has its own formulary, medical policies, required forms, and submission portal. A knee MRI might require PA from one payer with specific clinical criteria, and be auto-approved by another.

Submission Channel Chaos

Some payers accept electronic (X12 278). Many require proprietary web portals with CAPTCHAs. Others still rely on fax. ~20% of PAs still require a phone call. Architecture must be hybrid: FHIR API + RPA + fax + voice AI.

Clinical Judgment Requirement

PA is not pure paperwork. It requires reading the patient's chart, matching clinical evidence to payer-specific medical necessity criteria, extracting the right data points, and writing narrative justification. This is where LLMs matter.

Cost Per Transaction

Manual PA costs $10.97 per transaction (CAQH 2024). Fully electronic drops to $5.79. At 45 PAs/physician/week across 1.1M physicians, even small per-unit savings compound to billions.

The CMS-0057-F Mandate (Effective Jan 1, 2027)

What it requires: Payers must implement a FHIR R4-based Prior Authorization API that can: (a) identify all covered services requiring PA, (b) specify documentation requirements, (c) support electronic PA request/response exchange, and (d) communicate approval/denial with specific rationale. Decisions within 72 hours (urgent) or 7 calendar days (standard) — halving previous timeframes. Payers must publicly report PA metrics.

Who's covered: Medicare Advantage, state Medicaid/CHIP, Medicaid MCOs, QHP issuers on exchanges. Commercial/employer plans are NOT directly mandated.

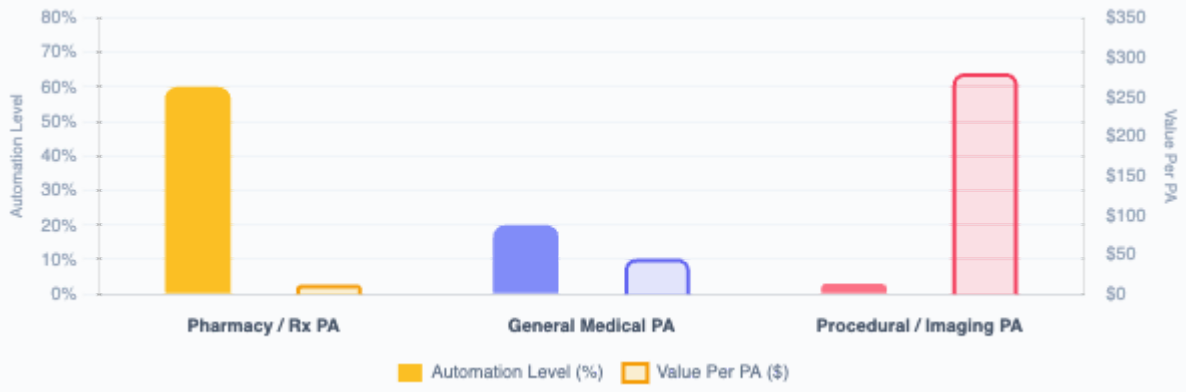
Critical signal: 50+ major insurers voluntarily committed in mid-2025 to simplifying PA, targeting 80% real-time electronic PA approvals by 2027.

SECTION 15

PA Market Segmentation & Competitive Tiers

The PA market splits into three distinct segments. Every current player is general-purpose. The procedural/imaging segment — highest-value, hardest, least-automated — is wide open.

PA Sub-Segments: Value vs. Automation Level



Pharmacy / Rx PA

Most automated. CoverMyMeds (McKesson) and Tandem ($1B valuation) dominate. Pharma-funded. Formulary-driven rules. ePA via Surescripts is standardized.

~60% automated

General Medical PA

Partially automated. Waystar, Rhyme, Myndshft, Silna cover broad PA types shallowly. Rules-based + AI bolt-ons. Portal navigation via RPA.

~20% automated

Procedural / Imaging PA

Least automated, highest value. MRIs, CT-guided procedures, surgery, spine instrumentation. Requires clinical reasoning from imaging data. No AI-native player exists.

~3% automated

Five-Tier Competitive Landscape

TIER	PLAYERS	APPROACH	LIMITATION
1. Legacy + AI Bolt-On	Waystar (NYSE: WAY), CoverMyMeds, Availity	Broad RCM / clearinghouse with PA as one module. Deep EHR + payer connections.	Not AI-native. Rules-based. Shallow PA automation.
2. AI-Native Scale	Cohere Health ($90M Series C), Tandem ($1B valuation)	Cohere: payer-side UM, 90% auto-approval. Tandem: Rx-only PA, pharma-funded.	Cohere sells to payers, not providers. Tandem is Rx-only.
3. AI-Native Specialists	Infinitus ($103M), Myndshft, Rhyme, Silna Health	Infinitus: voice AI for phone-based PA. Others: cloud-native end-to-end PA.	General-purpose across clinical domains. No imaging depth.
4. Vertical Specialists	Latent Health, Mandolin, Flow Auth, Tennr, Squad Health	Niche: specialty pharmacy, infusion, fax extraction, appeals.	Narrow scope. Not full PA automation.
5. EHR-Native	Epic (CRD/DTR/PAS), Oracle Health	Building PA workflows directly into EHRs. FHIR-based.	Basic submit-and-track. No AI clinical reasoning. No imaging awareness.

The critical gap: Every single player is general-purpose across clinical domains. None has deep domain expertise in surgical, procedural, or imaging workflows. That is the exact segment where PA is most painful, highest-stakes, and least automated. This is Slate's wedge.

SECTION 16

The Procedural/Imaging PA Opportunity

The imaging and procedural PA sub-segment is estimated at $5-10B TAM. A layer-by-layer competitive analysis confirms a validated, unoccupied whitespace.

Confirmed Market Gap: Layer-by-Layer Analysis

LAYER	WHO'S THERE	WHAT'S MISSING
Payer-side review (gatekeepers)	eviCore (Cigna), Carelon/AIM (Anthem), Stanson/ImagingAssure (Premier)	These define criteria. Not competitors — understanding their guidelines is the data moat.
Submission plumbing (logistics)	Infinx (strongest), Waystar, Rhyme	They automate portal navigation, form-filling, status checking. **No clinical reasoning.**
Clinical justification (the hard part)	**NOBODY**	**No company generates imaging-aware medical necessity arguments from actual imaging data.**
Imaging-native PA (data advantage)	**NOBODY**	**No company starts with DICOM data itself and works outward to PA submission.**
Closed-loop workflow	**NOBODY**	**No company connects imaging → surgical planning → PA submission → procedure execution.**

PA Agent Architecture: Four-Layer System

LAYER 1 — Clinical AI Engine

LLM reads clinical context + imaging data. Generates structured findings, measurements, narrative justification matching payer-specific medical necessity criteria. **Deepest moat.**

▼

LAYER 2 — Payer Rules Engine

Continuously updated database mapping CPT/HCPCS codes → payer-specific PA requirements, documentation needs, medical necessity criteria. Encodes eviCore, AIM, individual payer guidelines.

▼

LAYER 3 — Submission Engine

Post-2027: FHIR R4 PA API (Da Vinci PAS/CRD/DTR) for government payers. Pre-2027 + commercial: RPA portal bots + fax automation. Voice AI integration (Infinitus) for ~20% requiring phone calls.

▼

LAYER 4 — Appeals & Denial Management

When PAs are denied, generates appeals with peer-to-peer talking points grounded in actual imaging findings. AI-annotated imaging evidence included in appeal packages. This is where significant dollar value sits.

Payer-Side Gatekeepers (Obstacles, Not Competitors)

ENTITY	PARENT	ROLE
eviCore	Evernorth / Cigna	Manages radiology PA for dozens of health plans. Claims 1/3 of CTs and MRIs may be unnecessary.

ENTITY	PARENT	ROLE
Carelon / AIM	Elevance / Anthem	Similar RBM role. Payers frequently switch between eviCore and Carelon without provider notice.
Stanson / ImagingAssure	Premier Inc.	Radiology benefit management confirming medical necessity. Plus ReviewAssist for broader UM.

Regulatory tailwind: CMS launched the WISeR Model (2026-2031) — a six-year demonstration requiring prior authorization for radiology, interventional, and pain management procedures billed to Medicare Part B. Affects ASCs in 10 states (AZ, CA, FL, GA, MD, NY, OH, PA, TN, TX). This creates massive new PA volume specifically for imaging and procedural services.